

May 06, 2026

Ordering Provider: Dr. Sarah Chen, Internal Medicine
NPI: 1234567890

Patient: Sam Rodriguez | DOB: 09/04/1972 | Sex: M
Payer: United Healthcare | Plan: Choice Plus | Member ID: UHC9SR00287

REQUESTED PROCEDURE(S)

- CPT 73721 [PRIMARY] - MRI right knee without contrast
- ICD10 M23.231 [PRIMARY] - Derangement of anterior horn of medial meniscus due to old tear or injury, right knee
- ICD10 I10 - Essential (primary) hypertension
- ICD10 M25.461 - Effusion, right knee

MEDICAL NECESSITY NARRATIVE

Sam R. presents with derangement of the anterior horn of the medial meniscus due to old tear or injury of the right knee (ICD-10 M23.231) and right knee effusion (ICD-10 M25.461), for which MRI of the right knee without contrast (CPT 73721) is medically necessary. Conservative management has been attempted for at least six weeks, including ibuprofen 600 mg three times daily, without adequate symptom resolution. Physical examination reveals tenderness to palpation along the medial joint line and mild tenderness over the medial collateral ligament area, consistent with internal derangement. MRI imaging is required to further evaluate the suspected medial meniscal tear, rule out associated ligamentous or chondral pathology, and directly inform clinical management by determining whether operative versus non-operative treatment is appropriate.

ATTACHED DOCUMENTS

The following documents are attached:

- * Consult - 05/05/2026, Orthopedic Surgeon

Submitted by: Dr. Sarah Chen, Internal Medicine
NPI: 1234567890
Date: May 06, 2026

Provider Signature

CLINICAL NOTES

From encounter on 05/05/2026

Consult - Orthopedic Surgeon - 05/05/2026 10:45

- Apley's Compression Test: Positive with internal rotation.
- Lachman Test: Negative.
- Anterior/Posterior Drawer: Negative.
- Valgus/Varus Stress Test: Negative at 0 and 30 degrees.
- Patellar Grind: Negative.
- Ballottement: Mild fluid wave present.

Left Knee: Unremarkable.

Neurovascular: Distal pulses intact. Sensation intact. Motor 5/5 in bilateral lower extremities.

ASSESSMENT

Suspected medial meniscal tear, right knee. Physical exam findings are consistent with internal derangement of the right knee - positive McMurray test, medial joint line tenderness, and mild effusion - in a patient with a mechanism consistent with meniscal injury (twisting injury during athletic activity). Conservative measures have failed per patient report, with ongoing functional limitation after 4 months of symptoms.

Diagnosis: Internal derangement, right knee (M23.231 - Derangement of anterior horn of medial meniscus due to old tear or injury, right knee - working diagnosis pending MRI confirmation).

PLAN

1. Order MRI right knee without contrast (CPT 73721) - for further evaluation of suspected medial meniscal tear and to rule out associated ligamentous or chondral pathology. Imaging will directly change clinical management: if MRI confirms meniscal tear, patient will be a candidate for arthroscopic meniscal repair vs. partial meniscectomy; if negative, conservative management will be intensified.
2. Continue current anti-inflammatory regimen (naproxen 500 mg BID) while awaiting MRI results.
3. Refer for formal physical therapy evaluation in parallel (pending imaging).
4. Discuss surgical options with patient once MRI results reviewed.

ICD-10 codes: M23.231 (Derangement of anterior horn of medial meniscus due to old tear or injury, right knee) - primary working diagnosis

Signed: Dr. James Patel, MD | Orthopedic Surgery | NPI: 0987654321

Date/Time: 2026-05-05 10:45